

Access guide — tnbc-brca1-oligomet-liver-r7p3

How to access each therapy: trial contacts + manufacturer medical-info

Generated 2026-05-19

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	Carboplatin / cisplatin (platinum chemotherapy)	small_molecule	Standard of care + Off-label use	Both agents are FDA-approved across multiple solid tumor indications since the 1980s; generic for decades. Carboplatin is not specifically labeled for breast cancer, but it is on-compendium and ubiquitously used. Cisplatin similarly off-label in breast cancer.	Confirm renal function (CrCl for AUC dosing) and baseline hematology.
2	Olaparib (Lynparza)	small_molecule	Standard of care + Off-label use + Clinical trial	FDA-approved for germline BRCA1/2-mutant HER2-negative metastatic breast cancer (OlympiAD, Jan 2018) and for high-risk adjuvant germline BRCA1/2-mutant HER2-neg early breast cancer (OlympiA, Mar 2022). EMA-approved with comparable wording.	Order germline BRCA1 testing (saliva or blood) to resolve germline vs somatic — this is the single load-bearing question for on-label vs off-label prescribing.

3	Pembrolizumab + chemotherapy (KEYNOTE-355 1L mTNBC)	monoclonal_antibody	Standard of care + Off-label use + Clinical trial	FDA-approved Nov 2020 for 1L locally resectable or metastatic TNBC with PD-L1 CPS ≥ 10 by 22C3, in combination with chemotherapy (nab-paclitaxel, paclitaxel, or Also approved as tumor-agnostic monotherapy for TMB-H (≥ 10 mut/Mb by F1CDx) post-progression. EMA-approved with parallel wording.	Confirm PD-L1 CPS by 22C3 from the diagnostic biopsy — this is the only gating biomarker for the 1L pembro + chemo decision.
4	Stereotactic body / ablative radiotherapy (SBRT/SABR) and IR ablation for the solitary liver oligometastasis	other	Standard of care + Clinical trial	Not a drug — SBRT and IR ablation are procedural modalities, available at any academic radiation oncology / interventional radiology service. path, not regulatory.	Plan systemic therapy first; the consolidation question becomes relevant after 3-6 months of response.
5	Surgical resection of the intact primary tumor in de novo M1 breast cancer	other	Standard of care	Surgical procedure — not a drug. Available at any breast surgical oncology service.	Do not pursue primary surgery as part of the 1L systemic plan.
6	Talazoparib (Talzenna)	small_molecule	Standard of care + Off-label use + Clinical trial	FDA-approved for germline BRCA1/2-mutant HER2-negative locally advanced or metastatic breast cancer (EMBRACA, Oct 2018) and for HRR-mutant mCRPC + enzalutamide (TALAPRO-2, 2023). EMA-approved with similar mBC wording.	Resolve germline vs somatic BRCA1 testing alongside the olaparib decision — both drugs hinge on the same question.

7	Alpelisib (Piqray)	small_molecule	Off-label use	FDA-approved May 2019 for HR+/HER2-, PIK3CA-mutated advanced or metastatic breast cancer in combination with fulvestrant (SOLAR-1). Also approved as Vioice for PIK3CA-related overgrowth spectrum. No TNBC label; EPIK-B3 (alpelisib + nab-paclitaxel in PIK3CA-mut / PTEN-loss mTNBC) terminated early for slow accrual and a missed Part B1 ORR endpoint.	Do not pursue alpelisib in TNBC. The EPIK-B3 negative readout is the load-bearing fact.
8	Capivasertib (Truqap)	small_molecule	Off-label use	FDA-approved Nov 2023 for HR+/HER2- locally advanced or metastatic breast cancer with PIK3CA / AKT1 / PTEN alteration, in combination with fulvestrant after endocrine progression (CAPItello-291). No TNBC indication. CAPItello-290 (capivasertib + paclitaxel in 1L mTNBC) primary OS readout was negative at 2025 ESMO, including in the stratum.	Do not pursue capivasertib in TNBC. CAPItello-290 negative OS in the PIK3CA/AKT1/PTEN-altered stratum is the load-bearing evidence.

9	Datopotamab deruxtecan (Datroway, Dato-DXd)	ADC	Off-label use + Clinical trial	FDA-approved Jan 2025 for unresectable or metastatic HR+/HER2- breast cancer after prior endocrine and chemo Granted FDA Priority Review (Feb 2026) for 1L mTNBC ICI-ineligible based on TROPION-Breast02; approval expected mid-2026. EMA filing in parallel.	Wait on the PD-L1 CPS result before pursuing this drug — the access path branches on CPS ≥ 10 (TROPION-Breast05) vs CPS < 10 (TROPION-Breast02 label, compassionate use, or watchful waiting for the 2026 approval).
10	Inavolisib (Itovebi)	small_molecule	Off-label use + Clinical trial	FDA-approved Oct 2024 for endocrine-resistant, PIK3CA-mutated, HR+/HER2- locally advanced or metastatic breast cancer in combination with palbociclib + fulvestrant (INAVO120). No TNBC indication. EMA filing in parallel.	Do not pursue inavolisib in TNBC outside a trial.
11	Niraparib (Zejula)	small_molecule	Off-label use	FDA-approved for maintenance treatment of advanced epithelial ovarian, fallopian-tube, or primary peritoneal cancer after platinum response. No FDA approval in breast cancer; BRAVO phase 3 failed on central PFS and the program was not refilled in breast.	Default to olaparib or talazoparib before considering niraparib — same mechanism, on-label access, easier payer path.

12	Pembrolizumab monotherapy in TMB-H solid tumors (KEYNOTE-158)	monoclonal_antibody	Off-label use	FDA-approved Jun 2020 as tumor-agnostic monotherapy for unresectable or metastatic TMB-H (≥ 10 mut/Mb by FoundationOne CDx) solid tumors that have progressed on prior therapy and lack satisfactory alternatives. Breast cancer was NOT in the KEYNOTE-158 basket — application here is cross-tumor extrapolation under the agnostic label.	Confirm the TMB-14 call was made on FoundationOne CDx specifically — the label only recognizes that assay.
13	Sacituzumab govitecan (Trodelvy)	ADC	Off-label use + Clinical trial	FDA-approved for unresectable locally advanced or metastatic TNBC after ≥ 2 prior systemic therapies (≥ 1 in the metastatic setting) — the ASCENT label. Also approved 2L+ HR+/HER2-mBC after endocrine + 2 chemo lines. EMA-approved 2L+ mTNBC.	Reserve SG for the 2L decision. The 1L choice is pembrolizumab + chemo (CPS ≥ 10), Dato-DXd (CPS < 10 pending approval), or carboplatin + PARPi maintenance.

14	Adavosertib (AZD1775) and next-generation WEE1 inhibitors (azenosertib)	small_molecule	Clinical trial + Unavailable	Adavosertib has no FDA or EMA approval. AstraZeneca discontinued adavosertib in 2021 over tolerability across multiple programs. Azenosertib (Zentalis ZN-c3) is the next-generation successor, currently in phase 1/2/3 (ASPENOV phase 3 in cyclin-E1+ platinum-resistant ovarian cancer started May 2026). No breast indication trial open for azenosertib today.	Do not pursue adavosertib — no supply path.
15	Ceralasertib (AZD6738, ATR inhibitor) + olaparib	small_molecule	Clinical trial	Ceralasertib is investigational with no FDA or EMA approval. Olaparib carries the breast cancer indications separately (see olaparib row). The combination has no labeled use.	Do not pursue the combination outside a trial — no open protocol today.
16	Sacituzumab tirumotecan (sac-TMT, MK-2870)	ADC	Clinical trial	Investigational. No FDA or EMA approval. Filed in China for 2L+ TNBC (Kelun-Biotech, 2024); Merck holds ex-China rights and is running the global phase 3 program.	Wait on the PD-L1 CPS result. If CPS <10, contact the Merck trial sites line at 1-888-577-8839 (Trialsites@msd.com) for TroFuse-011 screening.
17	Samuraciclib (CT7001, CDK7 inhibitor)	small_molecule	Clinical trial	Investigational. FDA Fast Track designation for HR+/HER2- advanced breast cancer post-CDK4/6i and for locally advanced or metastatic TNBC. No approval. Carrick Therapeutics plans phase 3 initiation in 2026 (HR+ focus).	No open TNBC trial today; recheck Carrick's pipeline in 3-6 months.

18	ZEN-3694 (BET inhibitor) + pembrolizumab + nab-paclitaxel / + talazoparib	small_molecule	Clinical trial	Investigational. No FDA or EMA approval in any indication. Zenith Epigenetics holds the asset; NCI sponsors the TNBC combination trials.	Confirm PD-L1 CPS first — this trial requires PD-L1-negative.
19	Atezolizumab + nab-paclitaxel (IMpassion130 regimen)	monoclonal_antibody	Unavailable	US accelerated approval voluntarily withdrawn Aug 2021 after IMpassion131 failed confirmatory readout. EMA approval also withdrawn. Atezolizumab is FDA-approved for other indications (NSCLC, HCC, melanoma adjuvant) but the mTNBC indication is no longer labeled in the US or EU.	Do not pursue. The pembrolizumab + chemo row is the operational 1L ICI path.
20	Veliparib + carboplatin + paclitaxel (BROCADE3 / BrighTNess regimen)	small_molecule	Unavailable	No FDA or EMA approval in any indication. AbbVie discontinued the veliparib development program; the NCI CTEP / AbbVie expanded-access protocol (NCT03123211) has been listed as 'no longer available' since 2022, and clinical supply was terminated.	Do not pursue veliparib — there is no supply path.

21	Xentuzumab (BI 836845) — IGF-1/IGF-2 neutralising antibody, and the IGF-1R inhibitor class	monoclonal_antibody	Unavailable	No FDA or EMA approval in any indication. Boehringer Ingelheim has not pursued a marketing authorization. The phase 2 HR+ breast trial (XENERA-1) was negative and the program has been wound down for breast cancer; sponsor-driven trial closures since 2016-2017. Newer IGF-1R targeting assets (e.g., ganitumab, dalotuzumab, figitumumab) were all discontinued in oncology after class-wide negative readouts.	Do not pursue. No supply path exists.
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Standard of care (6)

1. Carboplatin / cisplatin (platinum chemotherapy)

Aliases: carboplatin, Paraplatin, cisplatin, Platinol

Modality: small_molecule · **Regulatory:** Both agents are FDA-approved across multiple solid tumor indications since the 1980s; generic for decades. Carboplatin is not specifically labeled for breast cancer, but it is on-compendium and ubiquitously used. Cisplatin similarly off-label in breast cancer. · **Guidelines:** NCCN category 1 / preferred for 1L mTNBC, with explicit support for platinum-containing regimens in germline BRCA-mut TNBC based on the TNT trial (ORR 68% carbo vs 33% docetaxel in BRCA-mut subset). NCCN Compendium covers the breast indication for both agents. · **Geographic scope:** Globally available. · **Verified:** 2026-05-19

Carboplatin is the cleanest, fastest standard-of-care lever for a BRCA1-mut TNBC patient. TNT showed double the ORR of docetaxel in the BRCA-mut subset (68% vs 33%), and the NCCN Compendium covers the use. The practical question is which backbone to pair it with — gemcitabine + carboplatin + pembrolizumab if CPS ≥ 10 , carboplatin + paclitaxel if CPS < 10 , or platinum-based induction followed by PARPi maintenance.

Next steps

1. Confirm renal function (CrCl for AUC dosing) and baseline hematology.
2. Plan the 1L backbone: gem/carbo + pembrolizumab (KEYNOTE-355) if CPS ≥ 10 ; carbo + paclitaxel (TNT-style) if CPS < 10 or platinum monotherapy preferred; carbo induction followed by olaparib maintenance is a defensible off-trial design for the BRCA1-mut phenotype (DORA template).
3. If platinum induction is the chosen path, plan for PARPi maintenance once response is established — see the

olaparib row.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05633654		recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Multiple generic manufacturers (Hospira/Pfizer, Teva, Sandoz, others)
- **Notes:** Generic; available through hospital and infusion pharmacies. No single sponsor medical-information line; product inquiries go through the prescribing institution's pharmacy.

Payer / coverage: Routine coverage across all US payers with NCCN Compendium support for breast cancer. No prior auth challenges. The TNT trial provides specific evidence for BRCA-mut TNBC platinum sensitivity, which strengthens documentation if needed.

Notes: TNT (Tutt 2018 Nat Med) is the cleanest BRCA-mut platinum-sensitivity evidence in advanced TNBC. CALGB 40603 supports the neoadjuvant pCR gain (60% vs 44%); BrighTNess 4-year confirmed carbo (not veliparib) drives the EFS benefit.

2. Olaparib (Lynparza)

Aliases: olaparib, Lynparza, AZD2281, KU-0059436

Modality: small_molecule · **Regulatory:** FDA-approved for germline BRCA1/2-mutant HER2-negative metastatic breast cancer (OlympiAD, Jan 2018) and for high-risk adjuvant germline BRCA1/2-mutant HER2-neg early breast cancer (OlympiA, Mar 2022). EMA-approved with comparable wording. · **Guidelines:** NCCN category 1 for previously treated recurrent or metastatic germline BRCA1/2-mutated HER2-negative breast cancer; listed on the NCCN Drugs & Biologics Compendium, which drives Medicare and commercial off-label coverage. · **Geographic scope:** Globally available — US, EU, UK, AU, Japan all carry the mBC label. · **Verified:** 2026-05-19

Olaparib is the cleanest near-term option once the germline-versus-somatic BRCA1 question is answered. If germline, it is on-label 1L for HER2-negative mBC and covered routinely. If somatic-only, the same prescription is off-label but mechanistically identical, and the NCCN Compendium plus the Stanford somatic-BRCA talazoparib trial make medical-exception coverage achievable.

Next steps

1. Order germline BRCA1 testing (saliva or blood) to resolve germline vs somatic — this is the single load-bearing question for on-label vs off-label prescribing.
2. If germline, write the script; AstraZeneca Access 360 (1-844-275-2360) handles benefits investigation and copay support.
3. If somatic-only, submit a medical-exception request citing the NCCN Compendium entry and the BRCA1-knockdown / HCC1937 preclinical data; expect 1-2 rounds of payer pushback.
4. Consider sequencing around the carboplatin-containing induction the patient is likely to start, with olaparib maintenance after response — the DORA design supports this even though DORA itself is not enrollable.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03821369	2	active not recruiting	no	—
NCT02734204	2	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://medicalinformation.astrazeneca-us.com/home/prescribing-information/lynpaenza-tablets.html>
- **Compassionate / expanded access:** <https://www.azandmeapp.com/>
- **Notes:** Access 360 case-management line for benefits investigations and prior-auth support: 1-844-275-2360 (1-844-ASK-A360). AZ&Me Application provides free product for uninsured patients meeting income criteria.

Payer / coverage: On-label for germline BRCA1/2 mBC, so Medicare and major commercial plans cover with prior auth requiring a germline test report. If the patient's BRCA1 mutation turns out to be somatic-only, the breast indication is off-label — payer pushback is likely; the standard workaround is to lean on the NCCN Compendium entry plus the Stanford NCT03990896 somatic-BRCA precedent and request medical-exception coverage.

Notes: OlympiAD remains the registrational evidence; OlympiA adjuvant approval added the post-surgical setting. The somatic-BRCA workaround is the active uncertainty for this case.

3. Pembrolizumab + chemotherapy (KEYNOTE-355 1L mTNBC)

Aliases: pembrolizumab, MK-3475, Keytruda, lambrolizumab

Modality: monoclonal_antibody · **Regulatory:** FDA-approved Nov 2020 for 1L locally recurrent unresectable or metastatic TNBC with PD-L1 CPS ≥ 10 by 22C3, in combination with chemotherapy (nab-paclitaxel, paclitaxel, or gemcitabine/carboplatin). Also approved as tumor-agnostic monotherapy for TMB-H (≥ 10 mut/Mb by F1CDx) post-progression. EMA-approved with parallel wording. · **Guidelines:** NCCN category 1 preferred regimen for 1L mTNBC PD-L1 CPS ≥ 10 . ESMO recommends I-A in the same population. · **Geographic scope:** Globally available — US, EU, UK, AU, Japan all carry the 1L mTNBC CPS ≥ 10 label. · **Verified:** 2026-05-19

Pembrolizumab + chemo is the 1L standard if the patient's PD-L1 CPS comes back ≥ 10 . Get the CPS result first — it is the single fork in the 1L decision tree. If CPS < 10 , this regimen is off-label and the access path shifts to Dato-DXd or carboplatin + PARPi.

Next steps

1. Confirm PD-L1 CPS by 22C3 from the diagnostic biopsy — this is the only gating biomarker for the 1L pembro + chemo decision.
2. If CPS ≥ 10 , write the script (pembro 200 mg q3w + investigator-choice chemo). Merck Access Program (1-855-257-3932) handles prior auth and reimbursement.
3. If CPS < 10 , do not pursue this regimen as 1L — see the Dato-DXd row (TROPION-Breast02 / compassionate use) and the carboplatin row.

4. TMB-H monotherapy indication (KEYNOTE-158) is technically open as cross-tumor extrapolation but requires prior progression and no satisfactory alternatives, so it is a 2L+ option not a 1L one for this patient.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06133864		recruiting	likely	—

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme
- **Med info phone:** 1-800-672-6372
- **Product info:** https://www.merck.com/product/usa/pi_circulars/k/keytruda/keytruda_pi.pdf
- **Compassionate / expanded access:** <https://www.merck.com/research/clinical-trials/expanded-access/>
- **Notes:** Merck Access Program: 1-855-257-3932 (Mon-Fri 8 AM-8 PM ET) handles benefits investigation, prior auth, and the KEY+YOU patient program (1-855-398-7832). Merck Patient Assistance Program for uninsured patients: 1-800-727-5400.

Payer / coverage: Routine Medicare and commercial coverage when PD-L1 CPS by 22C3 is documented ≥ 10 in the metastatic setting. The single load-bearing requirement is the CPS test result. If CPS < 10 , the regimen is off-label and payers will deny without an exception; in that case the practical pivot is to Dato-DXd (TROPION-Breast02) or trial enrollment.

Notes: KEYNOTE-355 final OS HR 0.73 (mOS 23.0 vs 16.1 mo in CPS ≥ 10) is the survival anchor. The TIL-3+ / TMB-14 phenotype reinforces the ICI rationale but the label is anchored on CPS, not TMB or TILs.

4. Stereotactic body / ablative radiotherapy (SBRT/SABR) and IR ablation for the solitary liver oligometastasis

Aliases: SBRT, SABR, stereotactic ablative body radiotherapy, thermal ablation, microwave ablation, radiofrequency ablation

Modality: other · **Regulatory:** Not a drug — SBRT and IR ablation are procedural modalities, available at any academic radiation oncology / interventional radiology service. Equipment-and-credentialing path, not regulatory. · **Guidelines:** NCCN does not endorse routine SBRT consolidation in oligometastatic breast cancer after the NRG-BR002 phase IIR negative readout. SABR-COMET (mixed histologies, including 18% breast) was numerically favorable but underpowered. Local therapy for a solitary liver lesion remains a case-by-case tumor-board call, not a guideline recommendation. · **Geographic scope:** SBRT / IR ablation broadly available at academic centers globally. MSK trial US-only. · **Verified:** 2026-05-19

Local therapy for the solitary liver oligomet is broadly available at academic centers — SBRT, microwave / radiofrequency ablation, or surgical metastasectomy are all on the table after systemic response is established. NRG-BR002 was negative for routine consolidation, so the operative framing is biology-driven tumor-board discussion rather than guideline-driven routine practice. The MSK phase 2 (NCT05534438) is the cleanest trial path if the team wants randomized-trial scaffolding.

Next steps

1. Plan systemic therapy first; the consolidation question becomes relevant after 3-6 months of response.
2. Refer to radiation oncology and interventional radiology early for joint planning — the 1.5 cm liver lesion is a candidate for SBRT or thermal ablation depending on the segment.
3. If trial scaffolding is desired, contact Dr. Amy Xu (xua@mskcc.org, 646-888-6863) at MSK for NCT05534438 screening once the patient has demonstrated systemic response.
4. Tumor board discussion is the right venue — NRG-BR002 makes 'routine SBRT for breast oligomets' inappropriate, but case-by-case consolidation for a single liver lesion in a young patient with curative-intent framing remains defensible.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05534438		recruiting	likely	Amy Xu, MD, PhD; xua@mskcc.org; 646-888-6863

Manufacturer / sponsor contact

- **Company:** Not applicable (procedural modality)
- **Notes:** Local therapy delivered at the institution. Access via radiation oncology / IR consultation.

Payer / coverage: SBRT for oligometastatic disease is covered by Medicare and most commercial payers with prior auth when there are ≤ 5 lesions and the primary is controlled. Coverage is generally easier when documented as consolidation after systemic response. IR thermal ablation has similar coverage patterns.

Notes: NRG-BR002 phase IIR PFS HR ~1.31 (favoring control) tempers the curative-intent framing. SABR-COMET 5-year OS update (Palma 2024) maintained the OS signal but with overlapping CIs. The clinical question is whether to proceed despite the negative breast-specific phase IIR — most centers will if biology and tumor board agree.

5. Surgical resection of the intact primary tumor in de novo M1 breast cancer

Aliases: primary tumor resection, locoregional therapy, mastectomy, breast-conserving surgery

Modality: other · **Regulatory:** Surgical procedure — not a drug. Available at any breast surgical oncology service. · **Guidelines:** NCCN does not recommend routine primary-tumor resection in de novo M1 breast cancer after the ECOG-ACRIN E2108 phase 3 OS-negative readout. Locoregional therapy is reserved for symptom control or after exceptional systemic response in selected patients. · **Geographic scope:** Globally available. · **Verified:** 2026-05-19

Primary-tumor resection is technically available but is no longer routine after E2108 showed no OS benefit. Practical use is reserved for symptom control (locoregional progression, ulceration, bleeding) or for selected patients with sustained oligometastatic conversion where the tumor board is comfortable reframing toward curative intent.

Next steps

1. Do not pursue primary surgery as part of the 1L systemic plan.
2. Revisit the question at 6-12 months if oligometastatic conversion is sustained and the liver lesion has been locally ablated — at that point the patient's biology may justify primary resection even though E2108 says no routine OS benefit.
3. Tumor board is the right venue for the eventual decision.

Manufacturer / sponsor contact

- **Company:** Not applicable (procedural modality)
- **Notes:** Delivered by breast surgical oncology + radiation oncology team.

Payer / coverage: Routine coverage when symptomatic or after sustained systemic response with curative-intent reframing. Not a payer obstacle.

Notes: E2108 (Khan 2022 JCO) 3-yr OS 68.4% vs 67.9%, HR 1.11 — definitive against routine resection but does not foreclose case-by-case decisions after exceptional response.

6. Talazoparib (Talzenna)

Aliases: talazoparib, Talzenna, BMN 673, BMN-673, MDV3800

Modality: small_molecule · **Regulatory:** FDA-approved for germline BRCA1/2-mutant HER2-negative locally advanced or metastatic breast cancer (EMBRACA, Oct 2018) and for HRR-mutant mCRPC + enzalutamide (TALAPRO-2, 2023). EMA-approved with similar mBC wording. · **Guidelines:** NCCN category 1 for previously treated recurrent or metastatic germline BRCA1/2-mutated HER2-negative breast cancer, alongside olaparib. Listed on the NCCN Drugs & Biologics Compendium. · **Geographic scope:** Globally available — US, EU, UK, AU all carry the mBC label. · **Verified:** 2026-05-19

Talazoparib is interchangeable with olaparib for the on-label germline-BRCA case and faces the same somatic-only off-label question. The differentiator is tolerability — talazoparib runs heavier on anemia and thrombocytopenia, so most teams default to olaparib unless once-daily dosing is preferred or olaparib is not tolerated. The Stanford NCT03990896 somatic-BRCA expansion is the active trial path if the germline test is negative.

Next steps

1. Resolve germline vs somatic BRCA1 testing alongside the olaparib decision — both drugs hinge on the same question.
2. If germline, run the olaparib-vs-talazoparib tolerability conversation; talazoparib's anemia profile may matter if the patient is starting from a borderline hemoglobin.
3. If somatic-only, contact Neelima Vidula (nvidula@mgh.harvard.edu) for NCT03990896 screening or pursue medical-exception coverage with NCCN Compendium support.
4. Pfizer Oncology Together (1-877-744-5675) handles benefits investigation and copay assistance once a prescription is written.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03920896		recruiting	likely	Neelima Vidula, MD; nvidula@mgh.harvard.edu; 617-724-4000

Manufacturer / sponsor contact

- **Company:** Pfizer Oncology
- **Med info phone:** 1-800-438-1985
- **Product info:** <https://www.pfizermedical.com/talzenna>
- **Compassionate / expanded access:** <https://www.pfizer.com/science/clinical-trials/expanded-access>
- **Notes:** Pfizer Oncology Together patient-support line: 1-877-744-5675 (financial assistance and reimbursement for prescribed product). Pfizer does not run a public TNBC compassionate-use protocol for talazoparib because the drug is commercially available.

Payer / coverage: Routine coverage when germline BRCA is documented. Somatic-only BRCA prescribing is off-label and faces the same payer pattern as olaparib — NCCN Compendium support is the lever for medical-exception approval. Hematologic toxicity (anemia ~39% G3+ on EMBRACA) makes talazoparib less first-pick than olaparib unless the patient has a reason to prefer once-daily dosing.

Notes: EMBRACA remains the pivotal evidence. The somatic-BRCA single-arm extension at Stanford / MGH is the cleanest open path for off-label conversion.

Off-label use (7)

7. Alpelisib (Piqray)

Aliases: alpelisib, Piqray, BYL719, Viojoice

Modality: small_molecule · **Regulatory:** FDA-approved May 2019 for HR+/HER2-, PIK3CA-mutated advanced or metastatic breast cancer in combination with fulvestrant (SOLAR-1). Also approved as Viojoice for PIK3CA-related overgrowth spectrum. No TNBC label; EPIK-B3 (alpelisib + nab-paclitaxel in PIK3CA-mut / PTEN-loss mTNBC) terminated early for slow accrual and a missed Part B1 ORR endpoint. · **Guidelines:** NCCN category 1 for HR+/HER2- PIK3CA-mut mBC. Not endorsed in TNBC NCCN guidelines or the compendium. · **Geographic scope:** Globally available for the on-label HR+ indication; off-label TNBC use anywhere requires payer override. · **Verified:** 2026-05-19

Alpelisib is approved for HR+/HER2- PIK3CA-mut mBC, not TNBC. The dedicated TNBC trial (EPIK-B3) terminated early after missing its primary ORR endpoint, which removes alpelisib from the practical TNBC option set even though the patient's PIK3CA-mut + PTEN-loss profile would have been the targeted subgroup. Off-label use is technically possible but the evidence base does not support it.

Next steps

1. Do not pursue alpelisib in TNBC. The EPIK-B3 negative readout is the load-bearing fact.
2. If the PI3K/AKT pathway is the priority, the inavolisib + eribulin TNBC trial (NCT07441512) is the clean

trial-only path — see that row.

3. If PIK3CA hotspot confirmation is still pending, get that resolved alongside the broader treatment decision; the hotspot call gates the ovarian / HR+ alpelisib precedent but does not change the TNBC story.

Manufacturer / sponsor contact

- **Company:** Novartis Oncology
- **Med info phone:** 1-888-669-6682
- **Product info:** <https://www.us.piqray.com/>
- **Compassionate / expanded access:**
<https://www.novartis.com/research-development/research-development-policies/managed-access-programs>
- **Notes:** Novartis Patient Navigator: 1-800-282-7630 for benefits investigation and copay support. Novartis runs managed-access programs case-by-case via the Managed Access portal.

Payer / coverage: Off-label in TNBC without compendium support, and the EPIK-B3 negative readout makes the evidence base actively unfavorable. Payers will almost certainly deny, and the medical-exception appeal is weakened by EPIK-B3. Hyperglycemia (the dose-limiting AE) further complicates the cost-benefit calculus.

Notes: EPIK-B3 Part B1 missed ORR threshold; Part B2 never initiated. Sponsor statement is the public record.

8. Capivasertib (Truqap)

Aliases: capivasertib, Truqap, AZD5363

Modality: small_molecule · **Regulatory:** FDA-approved Nov 2023 for HR+/HER2- locally advanced or metastatic breast cancer with PIK3CA / AKT1 / PTEN alteration, in combination with fulvestrant after endocrine progression (CAPItello-291). No TNBC indication. CAPItello-290 (capivasertib + paclitaxel in 1L mTNBC) primary OS readout was negative at 2025 ESMO, including in the PIK3CA/AKT1/PTEN-altered stratum. · **Guidelines:** NCCN category 1 for HR+/HER2- PIK3CA/AKT1/PTEN-altered post-endocrine. Not endorsed in TNBC NCCN guidelines; CAPItello-290 negative readout actively argues against TNBC use. · **Geographic scope:** Globally available for on-label HR+ use; off-label TNBC use anywhere requires payer override that is unlikely to succeed. · **Verified:** 2026-05-19

Capivasertib is HR+ on-label only. CAPItello-290 just read out negative in 1L mTNBC, including in the patient's likely benefit stratum (PIK3CA/AKT1/PTEN-altered) — that result effectively closes the off-label TNBC question. Do not pursue.

Next steps

1. Do not pursue capivasertib in TNBC. CAPItello-290 negative OS in the PIK3CA/AKT1/PTEN-altered stratum is the load-bearing evidence.
2. If the AKT-axis hypothesis is still compelling, the remaining option is ipatasertib (also negative in IPATunity130) — neither agent has a viable TNBC path today.

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-800-236-9933

- **Product info:**<https://medicalinformation.astrazeneca-us.com/home/prescribing-information/truqap.html>
 - **Compassionate / expanded access:**<https://www.azandmeapp.com/>
 - **Notes:** Same Access 360 / AZ&Me infrastructure as olaparib. Sponsor will steer TNBC inquiries toward the negative CAPItello-290 readout.
- Payer / coverage:** Off-label in TNBC without compendium support; the CAPItello-290 negative phase 3 readout makes the medical-exception appeal nearly unwinable. Hyperglycemia, diarrhea, and rash are the dose-limiting toxicities — adding payer pushback to a regimen with no efficacy gain in TNBC is not a winning trade.
- Notes:** CAPItello-290 OS HR 0.92 (overall) and 1.05 (PIK3CA/AKT1/PTEN-altered) reported at ESMO 2025; full publication Ann Oncol 2025. The trial is the cleanest available answer to the off-label TNBC question and the answer is no.

9. Datopotamab deruxtecan (Datroway, Dato-DXd)

Aliases: datopotamab deruxtecan, Dato-DXd, DS-1062, DS-1062a, Datroway, datopotamab deruxtecan-dlnk

Modality: ADC · **Regulatory:** FDA-approved Jan 2025 for unresectable or metastatic HR+/HER2- breast cancer after prior endocrine and chemo (TROPION-Breast01). Granted FDA Priority Review (Feb 2026) for 1L mTNBC ICI-ineligible based on TROPION-Breast02; approval expected mid-2026. EMA filing in parallel. · **Guidelines:** NCCN listed for HR+/HER2- 2L+ post-endocrine; not yet in NCCN TNBC guidelines pending the 2026 TNBC label expansion. The 1L mTNBC indication will likely land as category 1 once the label drops. · **Geographic scope:** TROPION-Breast05 recruiting globally including US, EU, AU. TROPION-Breast02 closed to enrollment. Compassionate use available case-by-case via Daiichi Sankyo. · **Verified:** 2026-05-19

Dato-DXd is in label-approval limbo for 1L mTNBC. The realistic path today is TROPION-Breast05 enrollment if PD-L1 CPS returns ≥ 10 , or a Daiichi Sankyo compassionate-use request if CPS < 10 and trial timing does not work. The full label expansion is expected mid-2026 and should make commercial 1L access routine within weeks of FDA action.

Next steps

1. Wait on the PD-L1 CPS result before pursuing this drug — the access path branches on CPS ≥ 10 (TROPION-Breast05) vs CPS < 10 (TROPION-Breast02 label, compassionate use, or watchful waiting for the 2026 approval).
2. If CPS ≥ 10 , contact the AstraZeneca Clinical Study Information Center at 1-877-240-9479 / information.center@astrazeneca.com for TROPION-Breast05 site screening.
3. If CPS < 10 and the team wants Dato-DXd now, submit a compassionate-use request via Daiichi Sankyo medical affairs (1-908-992-6400) — the framework exists and the TROPION-Breast02 data plus pending Priority Review give a clean evidence base.
4. Monitor the FDA action date for the TNBC indication (expected Q2-Q3 2026) — once approved, the off-label question disappears.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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NCT06133864	recruiting	likely	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com; 1-877-240-9479
NCT05374512	active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Daiichi Sankyo (developed with AstraZeneca)
- **Med info phone:** 1-908-992-6400
- **Product info:** <https://www.daiichisankyo.us/products/datroway/>
- **Compassionate / expanded access:** https://www.daiichisankyo.com/about_us/policies/compassionate_use_policy/
- **Notes:** Daiichi Sankyo runs a global compassionate-use program for Dato-DXd in life-threatening conditions when no trial slot is available and the patient cannot wait for label approval. Submission goes through the treating physician.

Payer / coverage: Currently covered on-label for HR+/HER2- 2L+. TNBC use today is off-label until the 1L mTNBC indication lands in mid-2026 — until then payer access requires either trial enrollment (TROPION-Breast05 for CPS ≥ 10) or a compassionate-use request through Daiichi Sankyo (CPS < 10 , ICI-ineligible, no trial slot).

Notes: FDA accepted the sBLA for 1L mTNBC ICI-ineligible under Priority Review in Feb 2026 — action date should land mid-2026.

10. Inavolisib (Itovebi)

Aliases: inavolisib, GDC-0077, Itovebi, RG6114

Modality: small_molecule · **Regulatory:** FDA-approved Oct 2024 for endocrine-resistant, PIK3CA-mutated, HR+/HER2- locally advanced or metastatic breast cancer in combination with palbociclib + fulvestrant (INAVO120). No TNBC indication. EMA filing in parallel. · **Guidelines:** NCCN listed for HR+/HER2- PIK3CA-mut endocrine-resistant mBC. Not endorsed in TNBC NCCN guidelines or compendium. · **Geographic scope:** On-label HR+ use available globally. TNBC trial mainland-China-only; not yet recruiting. · **Verified:** 2026-05-19

Inavolisib is HR+ on-label only and the TNBC trial sits in mainland China not yet recruiting. The PI3K/AKT pathway rationale for this patient is real, but inavolisib is not a practical reach today. If the team wants to act on the PIK3CA-mut + PTEN-loss biology, the cleaner option is to wait for the NCT07441512 trial to open or to consider the labeled HR+ regimen as a non-starter and look at capivasertib (also negative in TNBC, see that row).

Next steps

1. Do not pursue inavolisib in TNBC outside a trial.
2. If the team wants the trial path, NCT07441512 is mainland-China-only — confirm geography acceptance before any further work.
3. If PIK3CA hotspot confirmation is still pending, get it resolved; the call matters for downstream HR+ precedents even if not for TNBC use.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07421512		not yet recruiting	no	Hai Hu, MD; hjoyh@126.com; +86-15958192550

Manufacturer / sponsor contact

- **Company:** Genentech (Roche)
- **Med info phone:** 1-800-821-8590
- **Product info:** <https://www.itovebi-hcp.com/>
- **Compassionate / expanded access:** <https://www.gene.com/patients/expanded-access-and-compassionate-use>
- **Notes:** Genentech Medical Communications: 1-877-436-3683 for product-specific inquiries. Genentech runs an expanded-access framework but inavolisib is unlikely to be granted for TNBC outside the trial — the sponsor will steer toward INAVO120 evidence base in HR+ disease.

Payer / coverage: Off-label in TNBC, no compendium support, and the dedicated TNBC trial is in mainland China only. Payer denial is essentially guaranteed; the medical-exception path is weak because there is no published TNBC efficacy data.

Notes: INAVO120 PFS HR 0.43 in HR+ is the precedent; TNBC extrapolation is not supported by published efficacy data.

11. Niraparib (Zejula)

Aliases: niraparib, Zejula, MK-4827

Modality: small_molecule · **Regulatory:** FDA-approved for maintenance treatment of advanced epithelial ovarian, fallopian-tube, or primary peritoneal cancer after platinum response. No FDA approval in breast cancer; BRAVO phase 3 failed on central PFS and the program was not refiled in breast. · **Guidelines:** Not endorsed in NCCN breast cancer guidelines. NCCN Compendium does not list niraparib for breast cancer; off-label coverage in mBC will not have compendium support, which makes payer approval substantially harder than olaparib or talazoparib. · **Geographic scope:** Available wherever ovarian indication is approved; off-label use anywhere requires payer override. · **Verified:** 2026-05-19

Niraparib is approved in ovarian cancer but has no breast cancer label and no NCCN Compendium support — payer access is the bottleneck. The TOPACIO BRCA-mut TNBC ORR signal is mechanistically interesting, but the labeled PARP inhibitors (olaparib, talazoparib) cover the same ground with fewer payer obstacles, so niraparib rarely makes practical sense as a first reach in TNBC.

Next steps

1. Default to olaparib or talazoparib before considering niraparib — same mechanism, on-label access, easier payer path.
2. If pursuing niraparib for a specific reason (e.g., olaparib/talazoparib intolerance), GSK Medical Information (1-877-475-6448) can help with the medical-exception request packet.
3. Document the basis for off-label use — TOPACIO BRCA-mut ORR, prior PARPi failure / intolerance — for the

inevitable payer denial appeal.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02657289	1	completed	no	—

Manufacturer / sponsor contact

- **Company:** GSK
- **Med info phone:** 1-877-475-6448
- **Product info:** <https://zejula.com/>
- **Notes:** Together with GSK Oncology support line: 1-844-447-5662 for benefits investigation and copay assistance. Adverse-event reporting: 1-888-825-5249.

Payer / coverage: Off-label in breast cancer without NCCN Compendium support, so commercial and Medicare payers will typically deny. Possible appeals lever is the TOPACIO BRCA-mut TNBC ORR signal plus mechanistic equivalence to olaparib, but expect 2-3 rounds of denial. Most teams reach for labeled olaparib or talazoparib first and only consider niraparib if those agents fail.

Notes: BRAVO central-vs-local PFS discordance is the published interpretation caveat. GSK has not pursued a breast indication.

12. Pembrolizumab monotherapy in TMB-H solid tumors (KEYNOTE-158)

Aliases: pembrolizumab, MK-3475, Keytruda

Modality: monoclonal_antibody · **Regulatory:** FDA-approved Jun 2020 as tumor-agnostic monotherapy for unresectable or metastatic TMB-H (≥ 10 mut/Mb by FoundationOne CDx) solid tumors that have progressed on prior therapy and lack satisfactory alternatives. Breast cancer was NOT in the KEYNOTE-158 basket — application here is cross-tumor extrapolation under the agnostic label. · **Guidelines:** Tumor-agnostic pathway recognized in NCCN Biomarkers Compendium; breast cancer-specific use as monotherapy is not endorsed in NCCN breast guidelines and is uncommon practice. · **Geographic scope:** US tumor-agnostic indication is unique; EU and other agencies have not adopted the TMB-H tumor-agnostic wording. · **Verified:** 2026-05-19

The tumor-agnostic TMB-H label is real but mostly relevant as a 2L+ option, not 1L — the FDA wording restricts use to progression on prior therapy with no satisfactory alternatives. For a treatment-naïve mTNBC patient, pembro + chemo (KEYNOTE-355, on-label if CPS ≥ 10) is the operative path. Hold the TMB-H indication in reserve.

Next steps

1. Confirm the TMB-H call was made on FoundationOne CDx specifically — the label only recognizes that assay.
2. Do not pursue TMB-H monotherapy in 1L; the label does not support it and payers will deny.
3. Carry the TMB-H option forward as a 2L+ lever if standard options are exhausted, with documentation of progression on prior lines and no satisfactory alternatives.

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme
- **Med info phone:** 1-800-672-6372
- **Product info:** https://www.merck.com/product/usa/pi_circulars/k/keytruda/keytruda_pi.pdf
- **Notes:** Same Merck contact apparatus as the pembro + chemo row; the monotherapy indication runs through the same Access Program.

Payer / coverage: Tumor-agnostic indication is recognized but requires the TMB call to be from FoundationOne CDx specifically and the patient must have progressed on ≥ 1 prior line with no satisfactory alternatives. Payers will deny in the 1L setting even at TMB 14 mut/Mb because the label restricts use to post-progression with no alternatives. Becomes practical only in 2L+ when other options are exhausted.

Notes: Decision-relevant context, not a 1L action item. The CPS-anchored KEYNOTE-355 indication is the operational pembro path for this patient.

13. Sacituzumab govitecan (Trodelvy)

Aliases: sacituzumab govitecan, sacituzumab govitecan-hziy, IMMU-132, Trodelvy

Modality: ADC · **Regulatory:** FDA-approved for unresectable locally advanced or metastatic TNBC after ≥ 2 prior systemic therapies (≥ 1 in the metastatic setting) — the ASCENT label. Also approved 2L+ HR+/HER2- mBC after endocrine + 2 chemo lines. EMA-approved 2L+ mTNBC. · **Guidelines:** NCCN category 1 for 2L+ mTNBC. 1L use is off-label and off-guideline pending the ASCENT-04 label update (1L SG + pembrolizumab in PD-L1 CPS ≥ 10 — pivotal readout 2025; expected label update in 2026). · **Geographic scope:** Globally available — US, EU, UK, AU all carry the 2L+ mTNBC label. · **Verified:** 2026-05-19

Sacituzumab govitecan is the labeled 2L+ TNBC ADC. Not a 1L option today, but it is the most likely second-line drug after progression on a 1L pembro + chemo or carboplatin + PARPi regimen. The 1L expansion via ASCENT-04 (SG + pembrolizumab) is pending label.

Next steps

1. Reserve SG for the 2L decision. The 1L choice is pembrolizumab + chemo (CPS ≥ 10), Dato-DXd (CPS < 10 pending approval), or carboplatin + PARPi maintenance.
2. When the patient progresses, Gilead Oncology Support (1-844-876-3358) handles benefits investigation; expect routine 2L+ approval.
3. Watch the ASCENT-04 label update — if 1L SG + pembrolizumab gets PD-L1 CPS ≥ 10 indication, the 1L decision shifts.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05633654		recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Gilead Sciences
- **Med info phone:** 1-844-876-3358
- **Product info:** <https://www.trodelvyhcp.com/>
- **Compassionate / expanded access:**
<https://www.gilead.com/science/clinical-trials/compassionate-use-and-managed-access>
- **Notes:** Gilead Oncology Support: 1-844-876-3358 (1-844-TRODELVY), 9 AM-7 PM ET. Gilead Patient Assistance Program PAP enrollment fax: 1-833-851-4344. Compassionate-use route exists but Trodelvy is commercially available, so the typical path is on-label or off-label prescription, not EAP.

Payer / coverage: Routine coverage in 2L+ mTNBC. 1L use is off-label until the ASCENT-04 label update lands; commercial payers and Medicare will deny 1L prescriptions today unless the patient has a PD-L1 CPS <10 + ICI-ineligible profile and the prescriber documents the ASCENT-04 evidence base. NCCN Compendium does not yet list 1L use.

Notes: ASCENT-04 readout (Tolaney 2026, NEJM) showed PFS HR 0.65 for SG + pembro vs chemo + pembro in CPS >=10 1L — the label expansion is the near-term inflection point.

Clinical trial (5)

14. Adavosertib (AZD1775) and next-generation WEE1 inhibitors (azenosertib)

Aliases: adavosertib, AZD1775, MK-1775, azenosertib, ZN-c3

Modality: small_molecule · **Regulatory:** Adavosertib has no FDA or EMA approval. AstraZeneca discontinued adavosertib in 2021 over tolerability across multiple programs. Azenosertib (Zentalis ZN-c3) is the next-generation successor, currently in phase 1/2/3 (ASPENOVA phase 3 in cyclin-E1+ platinum-resistant ovarian cancer started May 2026). No breast indication trial open for azenosertib today. · **Guidelines:** Not in NCCN or ESMO guidelines. · **Geographic scope:** Adavosertib not available anywhere. Azenosertib trials currently US/EU in ovarian only; TNBC scope pending. · **Verified:** 2026-05-19

Adavosertib is gone; azenosertib is alive but its trials are not yet open in TNBC. The mechanistic rationale (TP53-mut + replication stress, BRCA1-mut + WEE1 inhibition) is preclinically strong but the clinical-evidence base is thin — the DFCI adavosertib + cisplatin TNBC trial (NCT03012477) missed its primary ORR endpoint and VIOLETTE closed the adavosertib + olaparib arm for toxicity. Watch for Zentalis to open a TNBC azenosertib trial in late 2026 or 2027.

Next steps

1. Do not pursue adavosertib — no supply path.
2. Azenosertib has no open TNBC trial today. Watch Zentalis' pipeline updates (<https://ir.zentalis.com/>) for a TNBC protocol — AACR 2026 preclinical data signals interest but no protocol is registered yet.
3. If a TNBC azenosertib trial opens during the patient's treatment course, contact Zentalis medical affairs (1-858-263-4400) to confirm eligibility.

Manufacturer / sponsor contact

- **Company:** AstraZeneca (adavosertib, discontinued); Zentalis Pharmaceuticals (azenosertib)

- **Med info phone:** 1-800-236-9933 (AstraZeneca); 1-858-263-4400 (Zentalis)
- **Product info:** <https://www.zentalis.com/our-research/azenosertib>
- **Notes:** Adavosertib supply discontinued. Azenosertib is investigational; the phase 3 ASPENOV trial (NCT06850781) is enrolling cyclin-E1+ platinum-resistant ovarian, not TNBC. Zentalis is signaling TNBC trials based on AACR 2026 preclinical data but no public protocol is open yet.

Payer / coverage: Not applicable — both agents are investigational or discontinued. Trial supply is the only route.

Notes: Included to close both the adavosertib (discontinued) and the azenosertib (not yet TNBC-open) questions explicitly. The user requested both — neither is a current option.

15. Ceralasertib (AZD6738, ATR inhibitor) + olaparib

Aliases: ceralasertib, AZD6738, olaparib, Lynparza

Modality: small_molecule · **Regulatory:** Ceralasertib is investigational with no FDA or EMA approval. Olaparib carries the breast cancer indications separately (see olaparib row). The combination has no labeled use. · **Guidelines:** Not in NCCN or ESMO guidelines. VIOLETTE phase 2 was negative for the combination over olaparib alone in BRCA-mut TNBC. · **Geographic scope:** Trial-only access historically; no current trial open in TNBC. · **Verified:** 2026-05-19

VIOLETTE closed without a positive readout, so there is no current path to ceralasertib + olaparib outside a trial, and the trial that mattered is now closed. The mechanistic rationale (BRCA1-mut + ATR inhibition restoring replication-fork dependence) is strong preclinically but the clinical translation has not arrived. The labeled olaparib monotherapy row carries the actionable BRCA1 axis for this patient.

Next steps

1. Do not pursue the combination outside a trial — no open protocol today.
2. Watch the AstraZeneca pipeline for a next-generation ATR+PARPi combination trial; the mechanistic case is alive even if VIOLETTE was negative.
3. Use the labeled olaparib pathway (see olaparib row) as the actionable BRCA1 axis.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03320847		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-800-236-9933
- **Product info:** <https://medicalinformation.astrazeneca-us.com/>
- **Notes:** Ceralasertib supply is restricted to active trials. AstraZeneca has not opened a breast-cancer-specific ATR+PARPi combination protocol after VIOLETTE.

Payer / coverage: Not applicable — investigational.

Notes: VIOLETTE BRCA-mut PFS HR ~0.81 (not significant) is the negative readout. The combination is on hold pending next-generation ATR inhibitors.

16. Sacituzumab tirumotecan (sac-TMT, MK-2870)

Aliases: sacituzumab tirumotecan, sac-TMT, SKB264, MK-2870

Modality: ADC · **Regulatory:** Investigational. No FDA or EMA approval. Filed in China for 2L+ TNBC (Kelun-Biotech, 2024); Merck holds ex-China rights and is running the global phase 3 program. · **Guidelines:** Not in NCCN or ESMO guidelines. · **Geographic scope:** Available in China commercially (Kelun-Biotech approval). Global trial in US, EU, AU via Merck. · **Verified:** 2026-05-19

Sac-TMT is investigational outside China. The only realistic US access path is TroFuse-011 enrollment, which is open and recruiting for the 1L PD-L1-low mTNBC patient. Trial supply is the access path; no off-label or compassionate route exists for US patients today.

Next steps

1. Wait on the PD-L1 CPS result. If CPS <10, contact the Merck trial sites line at 1-888-577-8839 (Trialsites@msd.com) for TroFuse-011 screening.
2. Compare against TROPION-Breast05 (CPS >=10) and TROPION-Breast02-anchored Dato-DXd label (CPS <10) before committing — these are the three TROP2-ADC paths in this case.
3. Confirm an open site that accepts the patient's geography (US-only acceptable for the TroFuse-011 sites we found).

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06841354		recruiting	likely	Merck Toll Free Number; Trialsites@msd.com; 1-888-577-8839

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme (partnered from Kelun-Biotech)
- **Med info phone:** 1-800-672-6372
- **Product info:** <https://www.merck.com/research/clinical-trials/>
- **Notes:** Investigational only; no commercial supply outside trial. Merck's general medical info line handles inquiries; no compassionate-use program is published because the agent is pre-approval and the global pivotal is recruiting.

Payer / coverage: Not applicable — investigational, supplied through the trial.

Notes: Different topoisomerase-I payload (belotecan derivative) than Dato-DXd's DXd — clinically distinct profile despite same TROP2 target.

17. Samuraciclib (CT7001, CDK7 inhibitor)

Aliases: samuraciclib, CT7001, ICEC0942

Modality: small_molecule · **Regulatory:** Investigational. FDA Fast Track designation for HR+/HER2- advanced breast cancer post-CDK4/6i and for locally advanced or metastatic TNBC. No approval. Carrick Therapeutics plans phase 3 initiation in 2026 (HR+ focus). · **Guidelines:** Not in NCCN or ESMO guidelines. · **Geographic scope:** Trial-only when trials open; no current TNBC trial. · **Verified:** 2026-05-19

Samuraciclib has Fast Track in TNBC but no open TNBC trial actively recruiting at scale, and the published TNBC data is abstract-only. The asset is alive but the practical access path for this patient today is essentially nonexistent. Watch Carrick's pipeline for a TNBC protocol opening in 2026.

Next steps

1. No open TNBC trial today; recheck Carrick's pipeline (<https://www.carricktherapeutics.com/pipeline/cdk7-inhibitor>) in 3-6 months.
2. If a TNBC trial opens, contact Carrick medical affairs through the corporate channel; they are responsive to MYC-driven TNBC inquiries given the mechanism.
3. The mechanistic case (CDK7 + MYC-amplification + basal subtype) is strong preclinically (Wang 2015 Cell; Patel 2018) but unrealized clinically — the patient's MYC-amp profile is the right phenotype, but the drug is not available yet.

Manufacturer / sponsor contact

- **Company:** Carrick Therapeutics
- **Med info phone:** +353-1-902-2700 (Dublin); +1-617-955-4949 (US)
- **Product info:** <https://www.carricktherapeutics.com/pipeline/cdk7-inhibitor>
- **Compassionate / expanded access:** <https://www.carricktherapeutics.com/contact>
- **Notes:** Carrick is a smaller biotech; medical-information inquiries go through the corporate contact. No published compassionate-use protocol for TNBC; samuraciclib TNBC clinical data has been abstract-only through 2025.

Payer / coverage: Not applicable — investigational.

Notes: Carrick's phase 3 plans are HR+ focused; TNBC follows. Watch ASCO / SABCS 2026 for protocol announcements.

18. ZEN-3694 (BET inhibitor) + pembrolizumab + nab-paclitaxel / + talazoparib

Aliases: ZEN-3694, ZEN003694

Modality: small_molecule · **Regulatory:** Investigational. No FDA or EMA approval in any indication. Zenith Epigenetics holds the asset; NCI sponsors the TNBC combination trials. · **Guidelines:** Not in NCCN or ESMO guidelines. · **Geographic scope:** US (NCI sites including Dana-Farber). Not open in EU or AU. · **Verified:** 2026-05-19

ZEN-3694 has the closest active trial to this patient's MYC-amplification + basal/TIL-rich axis. The NCT05422794

phase 1b at Dana-Farber is open and enrolls 1L PD-L1-low TNBC, which fits if the patient's PD-L1 CPS comes back <10. The Garrido-Castro contact at DFCI is the right outreach.

Next steps

1. Confirm PD-L1 CPS first — this trial requires PD-L1-negative.
2. If PD-L1-low, contact Dr. Garrido-Castro's group at Dana-Farber (877-442-3324) for screening; the MYC-amplified expansion cohort is pre-specified and the patient's MYC-amp call lands her there.
3. Compare against TROPION-Breast02 (Dato-DXd label expansion) and TroFuse-011 (sac-TMT) as the parallel PD-L1-low 1L options — three feature-targeting trials in the same population.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05422794	1b	recruiting	likely	Ana C. Garrido-Castro, MD (Dana-Farber); 877-442-3324

Manufacturer / sponsor contact

- **Company:** Zenith Epigenetics (NCI co-sponsor for the TNBC combination)
- **Med info phone:** 1-403-407-4090
- **Product info:** <https://www.zenithepigenetics.com/>
- **Notes:** Zenith publishes a corporate contact at info@zenithepigenetics.com; investigational supply is trial-only. NCI CTEP manages the protocol distribution for NCT05422794.

Payer / coverage: Not applicable — trial supply.

Notes: The ZEN-3694 + talazoparib BRCA-WT TNBC trial (NCT03901469) does NOT apply — that trial is BRCA-WT and this patient is BRCA1-mut. The applicable trial is NCT05422794 (ZEN-3694 + pembro + nab-paclitaxel).

Unavailable (3)

19. Atezolizumab + nab-paclitaxel (IMpassion130 regimen)

Aliases: atezolizumab, Tecentriq, MPDL3280A, RG7446

Modality: monoclonal_antibody · **Regulatory:** US accelerated approval voluntarily withdrawn Aug 2021 after IMpassion131 failed confirmatory readout. EMA approval also withdrawn. Atezolizumab is FDA-approved for other indications (NSCLC, HCC, melanoma adjuvant) but the mTNBC indication is no longer labeled in the US or EU. · **Guidelines:** Removed from NCCN TNBC guidelines after the US withdrawal. NCCN Compendium no longer lists the mTNBC indication. · **Geographic scope:** Indication withdrawn in US and EU. Some non-US, non-EU markets may still carry historical approval but this is not a realistic access path. · **Verified:** 2026-05-19

Functionally unavailable for this case. The IMpassion130 indication was withdrawn in 2021 after IMpassion131 failed confirmation, and the labeled 1L ICI in mTNBC is now pembrolizumab + chemo. Do not pursue atezolizumab for this patient.

Next steps

1. Do not pursue. The pembrolizumab + chemo row is the operational 1L ICI path.

Manufacturer / sponsor contact

- **Company:** Hoffmann-La Roche / Genentech
- **Med info phone:** 1-800-821-8590
- **Product info:** <https://www.gene.com/patients/medicines/tecentriq>
- **Notes:** Genentech Medical Communications handles the off-label inquiry, but they will confirm the indication was withdrawn and steer the call toward pembrolizumab + chemo. Atezolizumab is still available for other on-label uses, but not for 1L mTNBC.

Payer / coverage: No payer in the US will cover atezolizumab + nab-paclitaxel for 1L mTNBC — the indication is withdrawn and the alternative (pembrolizumab + chemo) is on-label with category 1 evidence.

Notes: Included to close the option explicitly. The IMpassion130 evidence is preserved in the dossier for context but the access path is closed.

20. Veliparib + carboplatin + paclitaxel (BROCADE3 / BrighTNess regimen)

Aliases: veliparib, ABT-888

Modality: small_molecule · **Regulatory:** No FDA or EMA approval in any indication. AbbVie discontinued the veliparib development program; the NCI CTEP / AbbVie expanded-access protocol (NCT03123211) has been listed as 'no longer available' since 2022, and clinical supply was terminated. · **Guidelines:** Not in NCCN guidelines or compendium. No guideline body recommends veliparib outside a clinical trial. · **Geographic scope:** Not available in any geography. · **Verified:** 2026-05-19

Veliparib is functionally unavailable. AbbVie discontinued the program after BROCADE3 missed OS and BrighTNess showed the carboplatin (not veliparib) was driving the pCR gain. The expanded-access protocol has been closed since 2022 and there is no compassionate path open today.

Next steps

1. Do not pursue veliparib — there is no supply path.
2. The BROCADE3 / BrighTNess takeaway is that the platinum component does the work in BRCA-mut TNBC, which translates into a carboplatin-containing 1L backbone (not a veliparib hunt).

Manufacturer / sponsor contact

- **Company:** AbbVie
- **Med info phone:** 1-800-633-9110
- **Notes:** AbbVie has wound down the veliparib program. Medical information will confirm no commercial or expanded-access supply is available.

Payer / coverage: Not applicable — no commercial product to bill.

Notes: The veliparib row is here to close the question, not to open a path. If a future BRCA-mut indication is filed by another sponsor, this row should be revisited.

21. Xentuzumab (BI 836845) — IGF-1/IGF-2 neutralising antibody, and the IGF-1R inhibitor class

Aliases: xentuzumab, BI 836845, BI-836845, BI836845

Modality: monoclonal_antibody · **Regulatory:** No FDA or EMA approval in any indication. Boehringer Ingelheim has not pursued a marketing authorization. The phase 2 HR+ breast trial (XENERA-1) was negative and the program has been wound down for breast cancer; sponsor-driven trial closures since 2016-2017. Newer IGF-1R targeting assets (e.g., ganitumab, dalotuzumab, figitumumab) were all discontinued in oncology after class-wide negative readouts. · **Guidelines:** Not in NCCN or ESMO guidelines. · **Geographic scope:** Not available in any geography. · **Verified:** 2026-05-19

Xentuzumab and the IGF-1R inhibitor class are not accessible today — sponsor programs have wound down across the class and no compassionate-use route exists. The IRS2-amplification axis in this patient's profile is real biology but there is no clinical lever for it right now. Watch for next-generation IGF-axis programs (none in registered phase 2/3 for breast cancer at the time of this verification).

Next steps

1. Do not pursue. No supply path exists.
2. Document the IRS2-amplification finding for future option re-evaluation; if a new IGF-axis program enters the clinic, this patient is a mechanistically clean candidate.

Manufacturer / sponsor contact

- **Company:** Boehringer Ingelheim
- **Med info phone:** 1-800-243-0127
- **Product info:** <https://www.boehringer-ingelheim.com/research-development/clinical-trials>
- **Notes:** Boehringer Medical Communications can confirm the xentuzumab breast cancer program is wound down; no compassionate-use protocol exists.

Payer / coverage: Not applicable.

Notes: XENERA-1 (HR+ breast, Schmid 2021 Breast Cancer Res) was negative; subsequent Boehringer breast trials closed. IGF-axis as a therapeutic class has had a flat decade in oncology.
